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Name: نوره عبد الجيد مهدي عبد الغنى National ID: 27308050103249	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
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المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Zoledronic acid for bone protection during hormonal therapy
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



٢٠٢٠/٠٧ ٢٧٣٠٨٠٥٠١٠٣٢٢٩
حاصلة على ديوم صنابع



الشي مسلمة
رمضان عبدالسلام عبدالحميد محمد
البيضاة سارية حتى ٢٠٢٧/٠٧/١٠





١٩٧٣/٠٥/٢٧

IL5344632

جمهورية مصر العربية
وزارة الداخلية

نوره
عبدالجيد مهدي عبدالقنى
مسكن رولان-بلوك ٤ : اسدخل ٢
المقطم - القاهرة

٢٧٣ - ٨ - ٥ - ١ - ٣٢ ٤٩

١٣٥٩

Patient Name : الأستاذة / نوره عبد الجيد مهدي	Request No. : 13008996
Patient No. : 27308050 ، 249	Request Date : 18/05/2024
Path No. : 1624/24	Verification Date : 05/06/2024
Sex : Female	Financial Category : ام دار السلام - نفقة الدولة
Age : 50 Y	Department : Out Patient
Referring Doc. :	IHC No. :
	Registered by : marwa mohamed yc

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Right breast mass.
 Nature of Specimen: Tru-cut biopsy.
 Technique:
 - Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
 - ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:
 * IPT (Positive control showed appropriate reactivity):
 - ER: Strong nuclear reaction, ... 10% of tumor cells (score 5/8).
 - PR: Faint nuclear reaction in 30% of tumor cells (score 4/8).
 - Her2: Strong complete membrane reaction in >10% of tumor cells (score 3+).
 - Ki67: 25% nuclear reaction of tumor cells.

Conclusion:
 Right breast mass, Tru-cut biopsy:
 Invasive duct carcinoma with lobular component, Grade II.
 IPT:
 - ER: Positive (score 5/8).
 - PR: Positive (score 4/8).
 - Her2: Positive (score 3+).
 - Ki67: Positive in 25% of tumor cells.

Comment:
 * Scoring system: "ER, PR"
 ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1,

Verified by :
 Dr.Rofayda Abdallah

Printing Date : 23/06/2024 11:53:25 Page 1 of 2 Printed By : marwa mohamed youss



غرفة رقم (4) - الدور الأول - مبنى رقم (444) - كورنيش النيل - مصر - القاهرة - مصر
 Room No. (4) - First Floor-Building No. 987- Nile Corniche - Min El Qadema - Cairo, Egypt

Patient Name: الأستاذة / مودة محمد السيد
Patient No.: 27308050103249
Path No.: 1624/24
Sex: Female
Age: 50 Y
Referring Doc.:

Request No.: 13008996
Request Date: 18/05/2024
Verification Date: 05/06/2024
Financial Category: أورام نازي السلام - نفقة التولية
Department: Out Patient

Registered by: marwa mohamed you

IMMUNOHISTOCHEMISTRY REPORT

1+, 2, between 1% and 10%; 3, between 11% and 33%, 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature

Prof. Dr. Amal Ahmed Hamedy
Dr. Rofaida Abdullah

Verified by:

Dr. Rofaida Abdullah

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Page 2 of 2

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غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor - Building No. 987 - Nile Corniche - Mizr El Qadima - Cairo, Egypt

Patient Name: Norah Abd Al Gayed Mahdy Abd Al Ghany	Date : 29/06/2024
Exam : C.T CHEST WITH CONTRAST	Age : 51 Year(s)
Medical ID: 1992/2024	Pat. ID: 65286

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN & PELVIS REVEALED: -**

1. Chest:

- Right breast skin thickening with few about three UIQ irregular mass lesions the largest is measuring 2.5 X 2 cm the other lesions are seen deeper and inseparable from the underlying pectoral muscle.
- Right axillary level I and II pathologically enlarged lymph nodes the largest measures 3.4 x 1.5 cm.
- Bilateral scattered tiny pulmonary nodules the largest is 3 mm at middle lung lobe lateral segment.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. Abdomen and pelvis:

- The liver is average in size with homogenous CT texture. No definite focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys

Patient Name: Norah Abd Al Gayed Mandy Abd Al Ghany	Date : 29/06/2024
Exam : C.T CHEST WITH CONTRAST	Age : 51 Year(s)
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- D1 lytic lesion measuring 6 mm.
- Tiny sclerotic foci seen at D10, D11, D12. D9 , D6.
- D3 and L2 lytic lesions with vertical striations mostly hemangiomas.

OPINION:

- *Reported case of breast cancer.*
- *Bilateral scattered pulmonary nodules likely benign in nature for follow up.*
- *D1 lytic lesion for bone scan.*
- *No definite hepatic metastatic deposits.*

DR. hala Sayed , MSc

Signature Dr. Mohamed Mohsen

29/06/2024 02:58 م

A-Patient ID:

1- Name : نور عبدالحجيد مبدى خدافى

Sex:

MDT form

3- Medical number:
Female

83214742

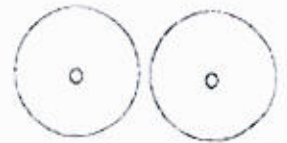
Old Med :

Date of MDT

1992-2024

2-ID number:

Patient Age : 51 Y



Family history

Family history

Phone

Phone

01156149097

BMI

BMI

REASON OF MDT

REASON OF MDT
POST-Operative

marital status

Marital status

NRS Score

NRS Score

GROUP

GROUP

Date of MDT

Date of MDT

Co morbidity

Co morbidity

appendectomy,

Menopausal
status

Menopausal status

Initial STAGE

Initial STAGE

22/01/202

Clinical data

Breast Imaging :

inflammatory rt breast cancer + skin thickness & edema +ve LN

Mets Workup :

ct CAP bil. ling benign nod& lytic lesion

BX

ER

PR

Her 2 Neu

SISH

Ki67

IDC

4/8

4/8

3

25

BX COMMENT

BX COMMENT

BX COMMENT

Her 2 - not COMMENT

SISH - COMMENT

with lobular componen

T

N

M

4b

1

Sonomammography

PATIENT HISTORY

pt received 6 TC+ 5 HP with good clinical response
last mdt:for Rt MRM as pre inial decision .

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

FOR ADJUVANT PHESGO UP TO 1 YEAR , PORT, AI FOR 5 YEARS CONSIDER EXTENDED

SIGNATURE

MEDICAL ONCOLOGY

MEDICAL ONCOLOGY

RADIOLOGY

RADIOLOGY

SURGERY

SURGERY

PATHOLOGY

PATHOLOGY

Psychiatric

Psychiatric

Nutrition

Nutrition